CLINICAL TRIAL PROTOCOL SUMMARY

Phase III Study of a Hypothetical Investigational Anti-Obesity Medicine

Protocol No.: OB-301-001 | Version 1.0 | 08 October 2026

Investigational Product: IMP-OB1 (hypothetical)

Sponsor	Example Therapeutics Pvt. Ltd. (hypothetical)
Study Phase	Phase III
Therapeutic area	Obesity / chronic weight management
Design	Multicentre, randomized, double-blind, placebo-controlled, parallel-group
Planned sample size	Approximately 100 vs 500 participants
Treatment duration	1 vs 52-week, with post-treatment safety follow-up

Important note: This is an academic/model protocol summary. It is not a regulator-approved protocol and must not be used to initiate a real clinical trial without study-specific scientific, medical, statistical, legal, regulatory, and Ethics Committee review.

1. Introduction and Regulatory Basis

Obesity is a long-term with many causes. It brings serious, health problems and hurts quality of life. Some people may get medicine for it along with changes in diet and exercise. This model protocol describes a pretend Phase III study. It tests IMP-OB1 for helping adults with obesity or overweight keep their weight down. These adults also have a health issue linked to weight. The study aims to collect good data on how well the drug works and how safe it is. At the same time, it protects the rights and well-being of everyone taking part.

The protocol follows Good Clinical Practice (GCP) rules. The main guide used worldwide right now is ICH E6(R3). That final rule was put in place on 06 January 2025. ICH E6(R3) puts focus on what matters for quality. It talks about managing risk in a way that fits each study. It also covers keeping participants safe, getting reliable results, and using technology and different study methods the right way. Later, on 03 June 2026, ICH added Annex 2. This annex covers extra GCP points for studies that use home-based parts, real-world elements, and data from everyday practice. [1]

In a study conducted in India, the researchers must follow rule from the New Drugs and Clinical Trials Rules, 2019 (NDCTR 2019). There are additional requirements from CDSCO, and oversight comes from a registered Ethics Committee. CDSCO says Ethics

Committees handle ethical review and oversight, this covers informed consent, how participants are recruited, protecting those who take part, recruitment, and any related compensation issues. [2] If IMP-OB1 meets the regulatory definition of a new drug or investigational product, the sponsor needs to get proper clinical-trial permission. They also must follow the applicable SUGAM/CDSCO process before exposing any participants to the investigational product. [3]

This study will also follow the World Medical Association Declaration of Helsinki, with the current official version is the 2024 revision. Before enrolling the first participant, the trial will be prospectively registered in the Clinical Trials Registry–India (CTRI) ahead of time. That matches CTRI's stated goal of registering clinical trials done in India before they begin. [4,5]

2. Study Objectives and Endpoints

Primary objective: To see how well IMP-OB1 works compared to a placebo for achieving clinically meaningful weight loss after 52 weeks of treatment when given alongside a standardized lifestyle program.

Primary endpoint: The percentage change from baseline in body weight from the start to at Week 52. The main estimand and how intercurrent events are handled – like stopping treatment or using rescue weight-management therapy, will be spelled out ahead of time in the statistical analysis plan (SAP).

Key secondary objectives will look at: (1) how many participants lose at least 5%, 10%, and 15% of their starting body weight; (2) changes in waist size; (3) changes in body-mass index (BMI); (4) selected measures of heart and metabolic health; (5) how participants rate their own quality of life; and (6) any side effects or safety issues that come up during treatment.

Exploratory objectives might cover changes in physical function, how well people stick to the treatment, certain metabolic markers in the blood, and specific subgroups picked ahead of time. These exploratory analyses will be clearly labelled as such – they won’t be treated as final proof of anything.

3. Methodology

3.1 Study Design

This will be a Phase III trial that is multicentre, randomized, double-blind, placebo-controlled, and uses a parallel-group design. Around 500 participants will be assigned in a 1:1 ratio to receive either IMP-OB1 or a placebo. A validated electronic system will create the randomization schedule, and when there is a scientific reason, it may be grouped by study site and baseline BMI category. Participants, investigators, people evaluating outcomes, and certain sponsor staff will not know who gets what unless an emergency requires unblinding. A 52-week treatment period comes after a screening

phase and before a post-treatment safety check lasting about 30 days. Study visits are set for screening, baseline and randomization, then Weeks 4, 8, 12, 20, 28, 40, and 52, ending with the safety follow-up. Remote contacts may be used only in cases where they are suitable, allowed, and backed by the risk assessment and any needed rules. 3.2 Participant Eligibility Key criteria for joining include adults from 18 to 75 years old who have a BMI of 30 kg/m^2 or higher. People with a BMI of 27 kg/m^2 or above may also qualify if they have at least one weight-related health condition listed ahead of time. They must also have a stable body weight or meet a protocol-defined weight pattern during screening. Willingness to stick with the study lifestyle programme and ability to understand and sign the informed consent form are required too. Key reasons for not letting someone in include having a medical condition that makes participation unsafe. People with uncontrolled heart, liver, kidney, hormone, or psychiatric issues that are serious enough will be excluded. Recent major heart or brain events that relate to the study drug are also a problem. A history of any disorder listed in the Investigator's Brochure that goes against participation will keep someone out. Pregnancy, breastfeeding if it is not allowed, or plans to become pregnant during the trial are also exclusion factors. Recent use of another investigational product or past bariatric surgery that could confuse the study results are disqualifying. Use of banned weight-loss medicines or procedures is not allowed. Each eligibility rule will be based on science, used the same way for everyone, and written down. Investigators will not enroll people when the risks outweigh the benefits or when another health issue would stop a proper evaluation of the study drug.3.3 Recruitment and Informed Consent

Recruitment may happen through investigator referrals, approved clinical databases, and ethically reviewed public ads. Materials used for recruitment must get approval from the Ethics Committee before they are put to use. People who might take part will get clear details about why the study is being done, what steps are involved, how long it will take, possible risks and benefits, other options besides joining in, privacy rules, payment arrangements if any apply, the fact that joining is voluntary, their right to leave the study, and who they can contact for more information. Written consent that is fully informed will be collected before any procedure that is part of the trial starts. During the discussion about consent, there will be enough time set aside for questions and thinking it over. This discussion will be handled by staff members who have been properly trained and given the task. Each participant will get a signed copy of their consent paperwork. New consent will be asked for when fresh information comes up that could change a person's decision to keep taking part, or if the Ethics Committee or a relevant authority requires it.

3.4 Intervention and Lifestyle Programme

IMP-OB1 and matching placebo will be manufactured, packaged, labelled, stored, dispensed, reconciled, and returned or destroyed according to applicable quality

requirements. The investigational product will be administered according to the protocol-defined dose-escalation and maintenance schedule, if applicable. Dose interruption or modification rules will be specified for tolerability and safety events.

All participants will receive a standardized lifestyle programme covering dietary quality, physical activity, behavioural strategies, and adherence support. The programme will be delivered consistently across sites using approved educational materials. Concomitant medications will be recorded throughout the trial, and prohibited medications or interventions will be defined prospectively.

3.5 Safety Monitoring

Safety assessments will include adverse-event review, serious adverse events (SAEs), physical examination as appropriate, vital signs, clinical laboratory testing, weight and other anthropometric measures, and additional assessments relevant to the pharmacology and known or potential risks of the investigational product. Safety findings will be evaluated continuously by investigators and the sponsor's medical safety function.

Adverse events will be assessed for seriousness, severity, expectedness, and relationship to study treatment in accordance with the protocol and applicable pharmacovigilance procedures. SAEs will be reported promptly through the sponsor's safety system and to the Ethics Committee and regulatory authorities within applicable timelines. Participants requiring medical care will be managed appropriately, and study treatment may be interrupted or discontinued as when required for safety.

A prespecified safety review process will identify emerging risks. Depending on the investigational product and risk profile, an independent data monitoring committee (DMC) may periodically review unblinded safety information under a written charter. Any stopping rules or protocol amendments prompted by safety findings will follow documented governance and regulatory procedures.

3.6 Data Management and Statistical Plan

Trial data will be captured via validated electronic systems or controlled source-data processes. Access will be role-based, with audit trails, edit checks, query management, controlled corrections, and documented data review. Essential records will be maintained so that the conduct of the trial conduct and the reliability of the results can be evaluated. ICH E6(R3) defines essential records as those documents and data that relate to the trial management and evaluation of GCP compliance and result reliability. [1]

Participant confidentiality will be protected by using coded identifiers, restricted access, and secure data transfer. Personal data will be processed in accordance with

applicable Indian law, institutional policies, and the approved consent framework. Data retention periods will be defined in the protocol and supporting plans.

The primary analysis will adhere to the prespecified SAP and estimand. The principal efficacy analysis will estimate the treatment difference in percentage change in body weight at Week 2 and provide an appropriate confidence interval and inferential procedure. Secondary endpoints, missing-data strategies, sensitivity analyses, subgroup analyses, multiplicity control, and analysis populations will be prospectively specified. The sample size will be justified by an expected treatment effect, variability, type I error, statistical power, and allowance for discontinuation.

4. Regulatory and Ethical Considerations

No participant will be enrolled before the required regulatory permissions, Ethics Committee approval, site readiness, and other applicable authorizations are in place. The sponsor will maintain controlled versions of the protocol, Investigator's Brochure, informed-consent documents, recruitment materials, safety information, randomization specifications, monitoring plan, data-management plan, and SAP.

The Ethics Committee will independently evaluate the scientific and ethical acceptability of the research, the risk-benefit profile, participant selection, informed-consent process, recruitment methods, confidentiality safeguards, and applicable compensation provisions. CDSCO specifically describes the Ethics Committee's role in protecting the rights, safety, and well-being of participants and overseeing informed consent, recruitment, and relevant compensation matters. [2]

The study will follow the 2024 Declaration of Helsinki. Risks and burdens must be minimized and reasonable in relation to anticipated benefits and the importance of the knowledge expected from the research. Participants will be informed of relevant new findings, may withdraw without penalty, and will not be disadvantaged in access to appropriate medical care because of refusal or withdrawal. Vulnerable individuals will receive additional safeguards where applicable. [5]

Because obesity can be associated with stigma, psychological distress, and socioeconomic barriers, recruitment and consent materials will use respectful, non-stigmatizing language. Participants will not be promised guaranteed weight loss or personal clinical benefit. Compensation, reimbursement, and any treatment for trial-related injury will follow applicable law, Ethics Committee approval, and the approved participant information and consent documents.

The sponsor will apply risk-proportionate quality management. Critical-to-quality factors may include participant eligibility, informed consent, randomization, investigational-product accountability, primary endpoint measurements, safety reporting, data integrity, and protocol compliance. Monitoring may combine centralized

review, targeted source-data verification, on-site activities, and audits as justified by the trial risk assessment.

Investigators will be appropriately qualified and trained. Delegated duties will be documented and assigned only to competent personnel. The sponsor and investigators will maintain conflict-of-interest disclosures and manage potential conflicts. Regulatory inspectors and authorized auditors will be given appropriate access to essential records and facilities in accordance with applicable requirements.

5. Quality Assurance, Records, and Trial Close-Out

The sponsor will maintain a Trial Master File and ensure that each site maintains appropriate essential records. Records will be retained for the legally and contractually required period and protected against unauthorized alteration, loss, or destruction. Computerized systems used for critical trial activities will be appropriately validated or qualified using a risk-based approach.

At study completion, the sponsor will complete data cleaning, query resolution, reconciliation of safety data, and database lock according to the data-management plan and SAP. The final clinical study report will describe participant disposition, protocol deviations, efficacy outcomes, safety findings, and interpretation. Investigational product will be reconciled and returned or destroyed according to approved procedures. Results will be reported transparently without compromising participant confidentiality.

6. Conclusion:

This protocol summary is intended to serve as a regulatory guide for a phase III clinical trial of a new anti-obesity investigational product. The key elements include a scientifically valid design, a reasonable choice of subjects, the relevance and credibility of clinical endpoints, and estimands, a thorough safety surveillance plan, valid data management practices, independent ethical oversight, informed consent of subjects, prospective trial registration of the trial, appropriate quality control measures, and reporting of result.

In the case of an obesity clinical trial, this protocol needs to be expanded upon to include specific statements related to the study drug such as the dose, endpoints, definitions, estimands, statistical assumptions and calculations, timing of procedures, laboratory tests, rescue-therapy rules, stopping criteria, pharmacokinetic or pharmacodynamic procedures where applicable, safety-monitoring governance, monitoring strategy, data-management specifications, investigational-product information, insurance, and compensation, and other documents specific to the participating countries. Such a protocol will be finalized upon the approval by the

relevant regulatory authorities, registered Ethics Committee, the sponsor's quality and medical departments, and other relevant bodies.

7. Regulatory References:

1. International Council for Harmonisation (ICH). ICH E6(R3) Guideline for Good Clinical Practice, Final Version adopted 06 January 2025; Annex 2 adopted 03 June 2026.

2. Central Drugs Standard Control Organization (CDSCO), Government of India. Ethics Committee requirements and regulatory materials under the New Drugs and Clinical Trials Rules, 2019.

3. CDSCO, Government of India. Investigational New Drugs / Clinical Trial application requirements under the New Drugs and Clinical Trials Rules, 2019.

4. Clinical Trials Registry–India (CTRI). Prospective registration requirements and Trial Registration Data Set.

5. World Medical Association (WMA). Declaration of Helsinki: Ethical Principles for Medical Research Involving Human Participants, 2024 revision.